VPS Lakeshore

KOCHI CAMPUS

Concept Design Brief

Lakeshore Hospital · Lakeshore Precision Medicine Centre

NH-66 Bypass, Nettoor, Maradu, Ernakulam, Kochi, Kerala 682040

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1. About VPS Lakeshore Hospital

Founded	1996 (incorporated); operational January 2003
Location	NH-66 Bypass, Nettoor, Maradu, Ernakulam, Kochi 682040
Scale	500+ beds · 100+ ICUs · 10 Operation Theatres · 30+ clinical departments
Accreditations	NABH
Firsts in Kerala	First liver transplant · First paediatric bone marrow transplant · First tri-modal ambulance (Limca Book of Records, 2004)
Patient Base	Local, national, and international — GCC diaspora, South Asia, Maldives, Africa

VPS Lakeshore Hospital is one of Kerala's largest tertiary care institutions, operated under the VPS Healthcare umbrella across Europe, the Middle East, and India. The hospital is nationally recognised as a centre of excellence in multi-organ transplantation, oncology, cardiac sciences, neurosciences, and orthopaedics.

Two decades of organic growth have produced a complex, multi-era facility at a critical inflection point: deep clinical capability and an established institutional brand set against infrastructure accumulated across successive phases of construction and regulation. This brief commissions a full-scale functional repositioning of the Kochi campus, aligned to a defined network architecture and a clear two-prototype clinical configuration.

2. Campus Configuration

The Kochi campus at Nettoor comprises two buildings — the Main Block (original building) and the New Block. Following the transformation programme, the two blocks will carry distinct institutional identities within the VPS Lakeshore network:

Main Block	New Block
Lakeshore Hospital	**Lakeshore Precision Medicine Centre**
Superspecialty Referral	Quaternary · Genomics-led
G + 10 floors	B2 + B1 + G + 10 floors

Lakeshore Hospital (Main Block) will be the super specialty referral institution — the clinical engine of the campus and the parent institution of the network. It will further strengthen Cardiac Sciences, Neurosciences, Maternal and Child Health, Orthopaedics, Gastroenterology, Nephrology, Emergency and Trauma, and high-acuity critical care.

Lakeshore Precision Medicine Centre (New Block) will be the quaternary genomics-led node — organised around five clinical programme verticals: Haemato-Oncology and BMT/CAR-T, Precision Oncology, Organ-Specific Transplantation, Rare Disease and Immunology, and Genomics, Prevention and Clinical Trials. Every programme in the building will be precision-defined. The New Block should carry a distinct entrance identity, dedicated patient reception, and ring-fenced clinical pathways separate from the Main Block.

The two blocks to be linked by a sterile service corridor at the surgical floor level, enabling phased transition of the transplant OT complex to the New Block upon commissioning of the LPMC high-sterility OT cluster.

3. Proposed Floor Configurations

The floor configurations below reflect a full clinical adjacency and infection control review. Key principles governing both stacks: BMT positive-pressure and AIIR negative-pressure isolation must not share lift cores or uncontrolled corridors; immunocompromised patients must be vertically separated from infectious disease wards; IVF requires independent HVAC; OT suites must be above public-facing OPD floors; and A&E must have a physically separate entrance and waiting area from elective OPD.

Floor Stack — New Block (Lakeshore Precision Medicine Centre)

Floor	Clinical & Functional Content
F10	Premium Inpatient — International, single rooms · Post-BMT/transplant step-down · Digital Command Centre · NRI and MVT accommodation
F9	BMT & Cellular Therapy — HEPA+ positive-pressure (allo/auto HSCT, CAR-T, GVHD) · Dedicated BMT nursing station · BMT Day Care · Pharmacy satellite *Positive-pressure — dedicated AHU and lift; no shared core with F8*
F8	Onco ICU · AIIR Isolation ×4 (negative-pressure) · Step-Down / HDU · Immunocompromised inpatient *Buffer floor — negative-pressure AIIR; BMT positive-pressure above directionally correct; separate AHU from F9*
F7	Haematology & Blood Disorders Ward · Complex Care Inpatient (rare tumours, sarcoma, neuro-oncology, neuromuscular) · Palliative & Symptom Management (dedicated sub-zone) *Neutral floor — buffer between F6 infusion and F8 AIIR*
F6	Chemotherapy Daycare · Complex Infusion (immunotherapy, bispecific antibodies, trial protocols) · Rare Disease Infusion (ERT, IVIG, factor concentrates) · Pharmacy satellite *Restricted access — no general OPD footfall; immunocompromised patients only*
F5	Clinical Trials Unit — Phase I monitoring bays (4), Phase II/III bays (10) · IMP Pharmacy · Bioinformatics workstations · Sponsor liaison *Adjacent to F6 infusion for IMP cold-chain and Phase I monitoring logistics*
F4	Comprehensive Liver Care & Hepatobiliary · GI Surgery · Medical Gastroenterology · Endoscopy & Bronchoscopy (dedicated decontam zone) · Gastro ICU (separated from endoscopy by clean corridor)
F3	Transplant OPD · Renal Sciences · GI & Hepatobiliary Institute OPD · MDT Clinic Pods · Transplant workup ·
F2	Neurosciences OPD · Rare Disease OPD · Genetic Counselling rooms · Neuromuscular OPD
F1	Onco Sciences & Haematology OPD · BMT OPD · Phlebotomy · Pharmacy
G	LPMC Reception & International Lounge · Precision Prevention Clinic (genomic risk, liquid biopsy, hereditary cancer screening) · Urgent Oncology / Toxicity Review (separate zone and entry) · OPD Billing · Admissions *Prevention clinic at Ground — well patients separated from treatment OPDs above*
B1	Advanced Radiology (MRI ×2, CT, SPECT-CT) · PET-CT with dedicated uptake / isolation rooms · MDT Boardrooms · Molecular Diagnostics & Genomics Lab · Apheresis unit *PET-CT below grade — superior shielding; dedicated uptake rooms isolate radioactive patients from public*
B2	Radiation Oncology (LINAC vault, HDR Brachytherapy, CyberKnife) · Dosimetry & Treatment Planning · Tumour Board Room · CSSD satellite · MRD back-office *CSSD collocated with heavy infrastructure; away from MDT meeting rooms (B1)*

Floor Stack — Main Block (Lakeshore Hospital)

Floor	Clinical & Functional Content
F10	Executive Floor — Suites, Deluxe, Single rooms · Dedicated Billing & Reception · VIP admission pathway *Quietest floor; furthest from A&E n oise and service traffic*
F9	Twin/Single Inpatient · Cardiac HDU · Immunocompromised Ward (dedicated AHU, restricted access) *Immunocompromised moved from F7 — now three floors above Infectious Disease wards; dedicated AHU; no shared corridor*
F8	General Inpatient Wards · Physiotherapy & Rehabilitation *Physiotherapy separated from Blood Bank (F7); general wards — no immunocompromised or infectious disease co-location*
F7	Blood Bank · Apheresis · Cord Store · IP Pharmacy *Separated from Physiotherapy; restricted-access zone.* IVF & Reproductive Medicine with *independent HVAC with VOC filtration; away from PICU infection load*
F6	Transplant ICUs — Liver TX, Renal TX, BMT support · Routine Laboratory · ICU Visiting · Dedicated lift for immunocompromised TX patients *Moved from F4 — one full buffer floor (F5 MICU/Cardiac ICU) above general critical care; dedicated lift mandatory*
F5	MICU · Cardiac ICU · IPD Billing *Buffer between Transplant ICU (F6) and Obstetrics/Paediatrics (F4); Dialysis removed from this floor*
F4	Paediatrics Ward · PICU · Reproductive Medicine (independent HVAC wing) · Microbiology, Pathology, Histopathology Lab *PICU separated from NICU (F3) — different floor eliminates shared-corridor infection risk; Lab reduces specimen transit from F3*
F3	Obstetrics · Labour & Delivery Suite · NICU (separate wing from PICU) · Neonatology *NICU and L&D co-located — correct; PICU moved to F4; no shared corridor between NICU and PICU*
F2	Main OT Suite (General, Cardiac, Neuro, Ortho, GI, Transplant interim) · DSA · Cath Lab · SICU · Anaesthesiology · Pre-op · Recovery *OT moved from F1 to F2 — above public-facing OPD/Admin floor; sterile zone separated from general public traffic*
F1	Specialist OPD · Dialysis · Daycare · Admin *OPD moved from Ground; Admin removed from OT floor; Dialysis accessible from wards by lift without ICU-floor transit*
G	A&E / Emergency (dedicated entrance, triage, resus — physically separated from elective) · Radiology (CT, MRI, Mammography, US, X-Ray, EEG, EMG/EP) · Pharmacy · Reception · Patient Services *A&E and elective OPD separated — different entrances, no shared waiting; Radiology at Ground for patient transport access*

4. Infrastructure Non-Negotiables

The following infrastructure elements are mandatory and must be specified in the design brief from concept stage. They cannot be retrofitted after structural decisions are made.

New Block — Lakeshore PMC

- BMT Suite (F9): dedicated AHU, HEPA H14 filtration, positive-pressure anteroom, dedicated lift — no shared elevator core with F8 AIIR floor
- AIIR Isolation (F8): negative-pressure rooms with anteroom, independent exhaust to atmosphere, dedicated nursing station — AHU completely separate from F9
- IVF (F3): independent HVAC with VOC filtration; no shared air handling with adjacent clinical areas
- PET-CT Uptake Rooms (B1): lead-lined, minimum 2 rooms, 90-minute hold protocol, no shared waiting with general radiology patients
- Endoscopy (F4): automated endoscope reprocessors, negative-pressure decontamination room, physically separated from Gastro ICU bay by clean corridor
- Pharmacy Compounding (B1/B2): ISO Class 5 cleanroom for chemo and biologic preparation, cold-chain storage, IMP vault for clinical trials
- Transplant OT Cluster: minimum 2 Class 10,000 rooms with laminar flow, perfusion-capable; sterile link corridor to Main Block F2 OT Suite
- Dirty/clean utility rooms, linen hold, and waste hold at every nursing floor (F6–F10)

Main Block — Lakeshore Hospital

- OT Suite (F2): full sterile zone separation from F1 public-facing floor; restricted-access staircase and lift core; no administrative traffic on F2
- Transplant ICU (F6): dedicated lift for immunocompromised patients — no shared core with general ward lifts; anteroom to each bay
- NICU (F3) and PICU (F4): physically separated wings with no shared corridor, nursing station, or air handling
- Immunocompromised Ward (F9): dedicated AHU zone, HEPA filtration, restricted access — no shared lift core with general ward floors
- A&E (Ground): entirely separate entrance, canopy, triage area, and waiting zone — no shared space with elective OPD reception
- Dirty/clean utility rooms, linen hold, and waste hold at every nursing floor (F3–F10)

5. Strategic Context & Design Philosophy

This brief commissions a full-scale functional repositioning of the VPS Lakeshore Kochi campus. The scope operates across three interlocking imperatives:

Clinical-First	Compliance-Driven	Experience-Led
Evidence-based adjacencies, flow logic, and clinical cluster design. Infection control embedded in architecture, not applied as an afterthought.	NBC, NABH, JCI, fire safety, biomedical waste, PwD norms — fully embedded from concept stage. Not a checklist; a design condition.	Dignified, calm, intuitive environments. Upscale but never ostentatious. The architectural language of competence, calm, and care.

The Five Design Imperatives

Every design decision must pass through the lens of these five principles:

- Flow — Zero confusion, zero delay. Legible circulation, separated movement streams, and intuitive spatial sequencing at every level.
- Time — Faster diagnosis, shorter stays. Parallel clinical workflows enabled by adjacency planning and diagnostic clustering.

- Continuity — Before, during, and after care: a seamless spatial and service continuum, from prevention through to post-discharge.
- Dignity — Privacy, calm, and respect as spatial conditions — embedded in layout, materials, acoustics, and light.
- Distinction — Differentiation through design intelligence: operational, spatial, and experiential.

> **Design Tone**
> Upscale. The architectural language of this facility is to communicate competence, calm, and care. Materials, lighting, acoustics, and wayfinding are to collectively produce an environment that is oriented, respectful, and safe.

6. Comprehensive Scope of Works

01 Regulatory Compliance & Code Alignment

All design and construction output to conform to current national and state regulatory standards across applicable codes and accreditation frameworks.

- Review and ensure compliance: National Building Code (NBC), Fire Safety Norms, NABH, JCI, and Biomedical Waste Management Rules
- Corridor width corrections — patient, bed, and fire evacuation compliant — across both blocks
- Fire exit planning: adequate number and distribution, pressurised staircases, fire-rated doors and compartmentalisation
- Smoke zoning and fire compartmentalisation
- Refuge areas and evacuation planning
- Accessibility compliance: barrier-free design (ramps, lifts, toilets), PwD norms throughout
- HVAC zoning for infection control — OTs, ICUs, BMT suite, AIIR isolation, and immunocompromised wards

02 Clinical Planning & Functional Reallocation

Departmental organisation to be structured around clinical adjacency logic, infection control requirements, and patient flow. The floor configurations in Section 3 provide the design team's working brief; the architect is expected to develop and refine these within the physical constraints of each building.

- Reorganisation of departments based on clinical adjacency, patient flow logic, and essential area requirements for waiting, OPD rooms, and bed areas
- Segregation of flows: Patient / Attendant / Staff / Clean Material / Biomedical Waste — across both blocks
- Vertical and horizontal circulation optimisation: dedicated patient lifts, service lifts, emergency lifts, BMT-dedicated lift (New Block F9)
- A&E repositioning: dedicated ambulance bay access, separate triage and resus from elective OPD
- ICU, OT, and diagnostics clustering for critical care efficiency
- Sterile vs non-sterile zoning — explicit delineation at every floor
- Sterile link corridor between New Block and Main Block at OT floor level (F2)

03 Floor-wise Accessibility & Circulation Planning

- Intuitive navigation across all floors: clear entry points, separated circulation for LPMC and Lakeshore Hospital
- Elimination of dead corridors and bottlenecks
- Spine corridors for major patient and material movement
- Logical vertical stacking as defined in Section 3
- Wayfinding strategy: signage, visual cues, colour-coded zoning — distinct visual identity for each block
- Accessibility for wheelchairs, stretchers, and emergency movement throughout

04 Departmental Rationalisation & Clean-Up

- Full audit of existing departments: redundant and underutilised areas across both blocks
- Removal of non-compliant and ad hoc spaces accumulated over successive development phases
- Standardisation of room sizes and clinical layouts
- Reconfiguration of OPD clusters — LPMC OPD floors (F1–F3, New Block); Lakeshore Hospital OPD (F1, Main Block)
- Optimisation of nursing stations and support areas at every floor

05 Aggregation of Fragmented Services

- Consolidation of diagnostics: Radiology, Pathology, PET-CT, Nuclear Medicine — per block, per floor as specified
- Creation of central support cores: CSSD (New Block B2, Main Block B1), Pharmacy, Stores
- Reduction of duplication across floors: doctor lounges, meeting rooms, seminar rooms
- Blood Bank and Apheresis consolidated in Main Block F7; Apheresis unit in New Block B1

06 Institute and Programme Zones

Each institute or programme zone to be established as a distinct, integrated zone with its own clinical identity, care pathway, and supporting infrastructure.

Lakeshore Precision Medicine Centre (New Block):

- Institute of Haemato-Oncology & BMT / CAR-T (F6–F9)
- Institute of Precision Oncology — Solid Tumours (F1, F6, B2)
- Organ Transplant Programme — Liver, Kidney, Multi-organ (F3–F4 OPD; F4 Hepatobiliary; linked to Main Block TX ICU F6 via sterile corridor)
- Rare Disease & Immunology Programme (F2, F6)
- Genomics, Prevention & Clinical Trials (G, F5, B1)

Lakeshore Hospital (Main Block):

- Institute of Cardiac Sciences (F2 Cath Lab/DSA, F5 Cardiac ICU, F9 Cardiac HDU)
- Institute of Neurosciences (F2 Neuro OT, F5 Neuro ICU — to be consolidated)
- Maternal & Child Health Institute (F3 OBG/NICU, F4 Paediatrics/PICU)
- Orthopaedics & Musculoskeletal Programme (F2 OT, F8 Ward)
- Gastroenterology & GI Surgery (F2 OT, F5 ICU buffer, F8 Ward)

07 Precision Medicine Centre — Specialist Requirements

The New Block carries specific infrastructure requirements that are distinct from conventional hospital design and must be addressed from concept stage:

- Genomics OPD and Genetic Counselling Rooms (F2): soundproofed, minimum 4 rooms, adjacent to waiting with privacy screening
- Precision Prevention Clinic (Ground): designed for well patients — aspirational, wellness-adjacent aesthetic, separated from treatment OPD waiting
- MDT Boardrooms (B1): minimum 2, video-linked to Delhi LPMC and network tumour boards; adjacent to radiology for direct image review
- Digital Command Centre (F10): real-time bed/OT/ICU dashboards, AI decision support, telemedicine hub — not patient-facing, positioned at top of clinical stack
- IMP Vault and Cold-Chain Storage (B1/B2): clinical trials investigational product storage, narcotics vault, cleanroom-grade biologic preparation
- Biobank Provision (B1): tissue/DNA/plasma storage, ICMR registry-linkable, adjacent to molecular diagnostics

08 International Patient Services

- New Block Ground: dedicated LPMC reception, International Lounge, concierge services — physically distinct from general patient entry

- Main Block F10: dedicated Executive Floor — suites, deluxe, single rooms, dedicated billing and reception, VIP admission pathway
- New Block F10: premium international inpatient — single rooms, post-transplant and post-BMT step-down recovery, hospitality-grade service standards
- Privacy-focused circulation: separate lift for Executive Floor; separate lift for BMT/immunocompromised; separate LPMC entrance
- Cultural customisation: food, language, concierge, visa, and travel services

09 Patient Experience & Environmental Design

- LPMC entrance — calm, precision-oriented arrival environment: reflecting clinical intelligence rather than conventional hospital aesthetics
- Main Block entrance — distinct from A&E; clear wayfinding to specialist OPD, radiology, pharmacy
- Waiting areas: acoustic control, natural light where possible, dignity-preserving layout
- Reduction of visible hospital clutter — clean lines, concealed service infrastructure
- Acoustic performance as a clinical and experiential priority: flooring, ceiling treatment, nurse call systems
- Family communication rooms at each ICU cluster
- Glass ICUs with privacy controls: clinical observation through glazed partitions, smart glass or privacy screening for dignity
- Food and beverage and retail — New Block B1 and Main Block Ground
- Digitally enabled experience: self check-in, queue management, indoor navigation

10 Infection Control & Safety Enhancements

- Pressure zoning: positive-pressure BMT (F9 New Block), negative-pressure AIIR (F8 New Block), negative-pressure isolation rooms (Main Block Transplant ICU F6)
- Dedicated AHU zones at every immunocompromised or high-infection-risk floor — no shared air handling across pressure zones
- Hand hygiene infrastructure at all critical points — clinical entry, bed head, nursing stations
- Biomedical waste flow segregation — dedicated waste lifts and holding areas per floor
- Surface material specification: anti-microbial finishes in ICUs, OTs, BMT suite, and isolation areas
- Negative pressure rooms in AIIR suite — 4 rooms minimum with anteroom and independent exhaust
- Endoscopy decontamination suite: automated reprocessors, negative-pressure room, no cross-flow with patient recovery area

11 MEP Upgradation

- HVAC redesign: OT-grade systems (Class 10,000 / Class 100 as required), BMT HEPA H14, AIIR negative-pressure, Transplant ICU anteroom pressurisation, IVF independent VOC-filtered supply
- Medical gas pipeline system audit and upgrade: MGPS, LMO/O_2 plant, vacuum, AGSS — across both blocks
- Electrical load assessment and redundancy planning: dual power supply, UPS, DG sets
- Fire alarm and suppression systems upgrade
- BMS (Building Management System) integration — both blocks on common platform
- Sterile link corridor MEP: pressure differentials, air handling, and clean supply to be specified for the inter-block link at F2

12 Digital & Smart Hospital Integration

- HIS/EMR integration planning — common platform across both blocks and network
- Smart nurse call systems
- Real-time patient tracking (RTLS)
- Telemedicine infrastructure — LPMC Digital Command Centre (F10 New Block) linked to Delhi LPMC and network tumour boards
- Digital signage and navigation: distinct visual identity per block
- Real-time bed and OT utilisation dashboards

- AI triage and clinical decision support integration points
- International language translation services
- Remote monitoring provision for post-discharge continuity

13 Phasing & Execution Strategy

The transformation programme is to be executed in a fully operational hospital environment. Phasing and sequencing are accordingly a primary design and planning responsibility.

- Phased renovation plan ensuring minimal disruption to ongoing clinical services — clinical continuity plans for each phase
- Temporary relocation planning: department-by-department sequencing with clinical team sign-off
- ICRA (Infection Control Risk Assessment) — mandatory for all construction phases in an operational hospital
- New Block: phasing to prioritise ground floor reception and basement diagnostics first, then OPD floors, then inpatient and critical care floors
- Main Block: phasing to preserve OT, ICU, and emergency function throughout — these floors to be last phased
- Sterile link corridor: phasing to be coordinated with LPMC transplant OT commissioning timeline
- Clear milestone-based execution timeline with clinical team, architects, engineers, and contractors

14 Branding, Identity & Positioning

- Two distinct institutional identities on one campus: Lakeshore Hospital (Main Block) and Lakeshore Precision Medicine Centre (New Block)
- Dedicated entrance signage, wayfinding, and reception identity per block — no shared main entrance
- LPMC visual identity: precision, intelligence, clinical science — materials and design language to reflect this
- Lakeshore Hospital visual identity: institutional authority, clinical depth, specialist referral — consistent with the established brand
- Consistent VPS Lakeshore brand across both blocks: navy and pink colour system, cross motif
- Institute and programme zone branding within each block
- NABH and JCI documentation alignment throughout

7. Distinctive Differentiators — Design Catalogue

The following are candidate differentiators for evaluation and integration by the design team. Each represents a specific opportunity to distinguish the Kochi campus through spatial, operational, or technological innovation.

Silent Waiting Acoustic design and digital queue management in place of audible name-calling. A measurable improvement in the waiting environment.	**Single Visit Diagnosis** Key clinical pathways resolved within a single visit through co-location of diagnostics and OPD, with same-day result protocols.
Glass ICUs with Privacy Controls Clinical observation maintained through glazed ICU partitions, with smart glass or privacy screening for patient and family dignity.	**Negative Pressure ICU Pods** All ICUs designed to negative pressure standards as a base condition — pandemic-ready by design.
AI-Assisted Reporting Zones Dedicated workstations for AI-supported radiology and pathology reporting, with infrastructure provision for machine learning integration.	**Central Command Centre** A purpose-designed Digital Command Centre for real-time bed management, patient flow monitoring, OT and ICU dashboards, and telemedicine linkage to the network.
Fast Track Surgical Pathways Same-day or next-day discharge protocols for cholecystectomies, joint replacements, and hernia repairs — embedded in the spatial design from the outset.	**Family Communication Rooms** Dedicated, enclosed rooms at each ICU cluster for clinical communication with families — privacy, acoustic separation, and dignity by design.

8. Concept Design Invitation

VPS Lakeshore Hospital and Research Centre invites qualified architecture and healthcare design practices to submit a Concept Design Response to this brief.

This is an open concept stage exercise. Submissions will be evaluated on the depth of clinical understanding, originality of spatial thinking, and the coherence of the design proposition in relation to the objectives set out in this document.

Concept Submission Requirements

Responses are expected to address the following areas. Submissions may be in PDF presentation format. Schematic and illustrative representation is appropriate at this stage; detailed construction drawings are not required.

01 Design Vision	A concise statement of the central design proposition for the campus — the governing principle that unifies spatial, clinical, and experiential intent across both blocks and the full scope of works. The response must address the two-identity configuration (Lakeshore Hospital and Lakeshore PMC) and how architectural language distinguishes and connects them.
02 OT Suite — Sample Layout	Schematic layout of a flagship OT suite illustrating adjacency relationships: sterile corridor, scrub zone, anaesthesia preparation, and recovery bay. Design intent to address material palette, clinical lighting, and HVAC zoning. The transplant OT cluster and its relationship to the sterile link corridor between blocks to be addressed.
03 ICU — Sample Layout	Concept layout for an ICU pod of 8–12 beds. Design to address patient visibility, nurse station sightlines, negative pressure provision, anteroom configuration, family interface, and acoustic performance. The balance between clinical observation and patient dignity is a key consideration.
04 LPMC Floor — Sample Layout	Illustrative floor plan for one LPMC floor — either the BMT suite (F9), the infusion/daycare floor (F6), or the Precision Prevention / OPD ground floor. Design to address restricted access zoning, pressure differentials, material palette, and patient wayfinding.
05 Inpatient Ward — Sample Floor	Illustrative floor plan for a single-speciality inpatient ward demonstrating bed configurations, nursing station placement, natural light strategy, family zone integration, and clean/dirty utility segregation. Room category differentiation — general, semi-private, and premium — to be indicated.
06 Distinguishing Design Elements	A curated set of 3–5 specific design interventions proposed to establish a distinctive identity for the Kochi campus. At least one intervention to address the LPMC identity specifically. Precedents, references, and schematic representations may be included.
07 Phasing Strategy	A high-level phasing narrative for an operational hospital environment, addressing clinical continuity, ICRA protocols, the sequencing of both blocks, and the commissioning of the sterile inter-block link corridor.

Evaluation Framework

Submissions will be assessed across five dimensions: (1) Fidelity to brief — depth of engagement with the clinical and operational objectives stated herein. (2) Conceptual rigour — coherence and originality of the design proposition. (3) Clinical credibility — evidence of understanding of hospital operational logic and regulatory requirements. (4) Spatial intelligence — quality of reasoning demonstrated in sample layouts. (5) Execution viability — plausibility of the phasing and delivery approach in an active hospital context.

Submission Parameters

- Format: PDF presentation, not exceeding 20 pages, exclusive of practice profile
- Sample layouts at schematic / concept level of development
- Precedent references and illustrative images to be appropriately sourced
- Practice profile to include lead designer, relevant hospital design experience, and completed projects
- All submissions to be treated in strict confidence